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Name: جلسن عبد المنعم محمد جمعه National ID: 25512010101986	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest and abdomen
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name: Jasnen Abdel Moneim Mohammed Gomaa
Date: 2026-08-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left breast lump

Findings:

left **breast.**

- Scattered fibroglandular densities. (ACR b)
- UIQ, two irregular speculated high density mass lesion with distortion
- **Ultrasound** demonstrated shows two irregular hypo echoic solid lesion seen at 7 o'clock zone C , 7 mm from the skin , measuring 14 x15 mm and at 8 o'clock zone b , 10 mm from the skin measuring 15 x12 mm . They show internal vascularity in color doppler
- Normal skin/nipple/areola complex.
- Single pathological axillary lymph node measuring 2.8 x 1.5 cm with faint central hilum

right **breast.**

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion: -

- left **breast (BIRADS 5): two suspicious looking lesion with ipsilateral single pathological axillary node for biopsy**
- right **breast (BIRADS 1): Normal study.**

Dr farah elshobaky

جمهورية مصر العربية

مطابق طبق الهوية



جنس

عبد المنعم محمد جمعه

١٠ ش مصلح مفرج

المسكين - القاهرة

١٩٥٥/١٢

٢٥٥١٢.١.١.١٩٨٦

LB6633314





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Dr farah elshobaky



Gustave Roussy International Egypt- Pathology Lab
جوستاف روصي - مصر
مركز محمد عبد المنعم الدولي للسرطان



Patient Name	جوليس عبد المنعم	Case No.	358687
Sex/Age	Female 70 Y	Patient No.	25996
Registered Date	17/8/2026 02:18:14 PM	Referred By Dr.	Interventional Radiology
Validation Date	07/9/2026 02:27:09 PM	Branch	Gustave Roussy
Reporting Date	16/9/2026 12:47:42 PM	Pathology No.	2612/26

Pathology Report

Clinical Data: Left breast LIQ mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 17/08/2026, at 2:05 pm. Cold ischemia time is 20 seconds.

Gross Examination:

-Three grey, white tissue cores, the longest one measured 1.0 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 1).
- DCIS: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 99% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 99% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 30%.

Conclusion:

Left breast mass, Tru-cut biopsy:

-Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Low (score 1+).
- Ki67%: 30%.



Gustave Roussy International Egypt Pathology Lab
جوستاف روسي انترنشنل - مصر معهد الباثولوجي



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****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim, MD.

٢٠١٩/١١/١٠

Advanced Clinical Exam

جولستان للخدمات الصحية

١١٤٤هـ

مراجعة

الذي الأمين

الذي الأيسر

صالح

Dis

6741

طبيعي	تراوح أو تغيير في اتجاه الحلمة	طبيعي	تراوح أو تغيير في اتجاه الحلمة
بروز الأوردة	تقرحات بالجذبة أو الحلمة	بروز الأوردة	تقرحات بالجذبة أو الحلمة
تغير في الشكل أو الحجم	افرازات غير طبيعية	تغير في الشكل أو الحجم	افرازات غير طبيعية
حدوث تورف	الشمعور بأورام بالأبط	حدوث تورف	الشمعور بأورام بالأبط
طاقح	الشمعور بسمك عام	طاقح	الشمعور بسمك عام
تواجد بالجذبة	ألم بالثدي	تواجد بالجذبة	ألم بالثدي
تحول الجذبة لسا يشبه قعر البركان	ورم محسوس	تحول الجذبة لسا يشبه قعر البركان	ورم محسوس
الإحصار	تم إلتصاق الثدي	الإحصار	تم إلتصاق الثدي

Maximum mass dimension (mm):

Side:

Right
left
Bilateral

Quadrant:

OUOQ

OUOQ

OUOQ

OUOQ

Central and retroareolar

Axilla

Imaging study requested:

Sonography

Mammosonography

OMRI breasts

lt breast 109

lesion (??)

for

BBSM

د مسأله و اليا

70-41

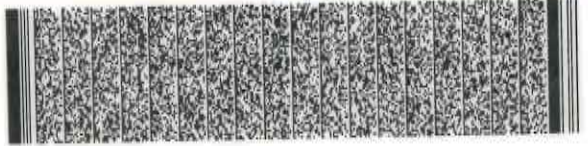
جمهورية مصر العربية
بطاقة تحليل الشخصية

جلسن
عبد المنعم محمد جمعه
١٠ ش مصلح مفرج
الهسائين - القاهرة

١٩٥٥/١٢  ٢٥٥ ١٢.١.١.١٩ ٨٦

LB6633314

٢٠٢٥/٠٣ ٢٥٥١٢٠١٠١٠١٩٨٦
ربة منزل
أنثى
مسلمة
أرملة
البطاقة سارية حتى ٢٠٣٢/٠٣/١٢ ١



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٢٠١٩.١٠.١٠.٢٠١٩

Advanced Clinical Exam

جولستان للعلوم

حالة الثدي الأيسر

١١٤٤
مراجعة

Dis

6741

طبيعي	تراجع أو تغيير في اتجاه الحلمة	طبيعي	تراجع أو تغيير في اتجاه الحلمة
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الإحصار	تم إستئصال الثدي	الإحصار	تم إستئصال الثدي

• Maximum mass dimension (mm):

• Side:

• Right
• left
• Bilateral

• Quadrant:

• UOQ
• UIQ
• OLOQ
• OLIQ

• Central and retroareolar
• Axilla

• Imaging study requested:

• Sonography
• Mammosonography
• MRI breasts

lt breast 109

for

BBSM

د. محمد الوائلي

70-
-ft

Radiological exam

Breast Density	A	B	C	D
Mammography	Normal			
	Mass			
	Benign calcifications			
	Malignant calcifications			
	Distortion			
	Asymmetry			
	N/A			
Ultrasound	Normal			
	Solid			
	Cyst			
	Complex cyst			
	Inflammatory changes			
	Dilated duct			
	Skin changes			
	Nipple involvement			
	Chest wall			
SITE	UOQ			
	UIQ			
	LOQ			
	LIQ			
	Retro-areolar			
	Axillary tail			
	N/A			
Size (largest)	No mass (T0)			
	0-2 cm (T1)			
	2-5 cm (T2)			
	>5cm (T3)			
	Could not be measured (Tx)			
	N/A			
Number	Single			
	Multifocal			
	Multicentric			
	N/A			
Axillary nodes	Non specific			
	Indeterminate			
	Pathological			
	N/A			
BIRADS	1 - 2 - 3 - 4 - 5 - 6			
Biopsy				
Remarks				

7 5
Dr. Fahn